

**Laboratory Investigation Report**

Patient Name	: Priyanka Adarsh	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 34 Y O M O D /F	OP/IP No	: OP/97622
Max ID/Mobile	: 437422/8752005200	Collection Date/Time	: 17/Aug/2021 01:47PM
Lab ID	: 1266082100401	Receiving Date	: 17/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 18/Aug/2021

Clinical Biochemistry**Blood Sugar (Random), Fluoride Plasma***

Date	17/Aug/2021 01:47PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	64.01	mg/dl	74 - 140

Please correlate with clinical History

Advised: Repeat Sample

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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SIN No: B2B845145, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752
Max Lab - A Division of Max Healthcare Institute Ltd.

The : Max Super Specialty Hospital, Saket (West Block)
1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

CIN No.: L72200MH2001PLC322854

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Hematology

CBC (Complete Blood Count)*, Whole Blood EDTA

Date	17/Aug/2021 01:47PM	Unit	Bio Ref Interval
Haemoglobin	11.41	g/dl	12.0 - 15.0
Packed Cell, Volume Calculated	34.9	%	36-46
Total Leucocyte Count (TLC)	8.88	10~9/L	4.0-10.0
Electrical Impedance			
RBC Count	4.02	10~12/L	3.8-4.8
Electrical Impedance			
MCV	86.8	fL	83-101
Electrical Impedance			
MCH	28.4	pg	27-32
Calculated			
MCHC	32.7	g/dl	31.5-34.5
Calculated			
Platelet Count	293.7	10~9/L	150-410
Electrical Impedance			
MPV	9.81	fL	7.8-11.2
Calculated			
RDW	14.7	%	11.5-14.5
Calculated			

Differential Cell Count

VCS / Light Microscopy

Neutrophils	66	%	40-80
Lymphocytes	27	%	20-40
Monocytes	05	%	2-10
Eosinophils	02	%	1-6
Basophils	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	5.86	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.4	10~9/L	1.0-3.0
Absolute Monocyte Count	0.44	10~9/L	0.2-1.0



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Hematology

Absolute Eosinophil Count 0.18 10~9/L 0.02-0.5

Kindly correlate with clinical findings

*** End Of Report ***


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Immunoassay

Total - Thyroid Profile*, Serum

Date	17/Aug/2021 01:45PM	Unit	Bio Ref Interval
T3 (Total) CLIA	1.0	ng/mL	0.87-1.78
T4 (Total) CLIA	9.68	µg/dL	6.09-12.23
TSH Chemiluminescence	3.96	uIU/ml	0.34-5.6

Comment

Parameter	Unit	Cord Blood	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	2.3 - 13.2	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.

Decreased in primary Hyperthyroidism

Total Thyroid Profile : (Thyroid Function Test, TFT)

T3 (Total), Triiodothyronine

Increase in Hyperthyroidism, and T3 toxicosis,

Decreased in hypothyroidism, states with decreased TBG, and acute and subacute non thyroidal illness

T4(Total) Thyroxine

Increased in Hyperthyroidism, states with increased TBG, Thyrotoxicosis

Decreased in Hyperthyroidism, states with decreased TBG and Strenuous exercise

TSH, Serum : Thyrotropin(Thyroid Stimulating Hormone)

Increased in primary Hypothyroidism.

Decreased in primary Hyperthyroidism.

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am

and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence

time of the day has influence on the measured serum TSH concentrations.

TSH assay is strandized to the 3rd generation for human TSH.

The Cyclical variations may be quite large; therefore the timing of specimen collection must be strictly controlled.

Advise : Kindly do Thyroid Profile/TSH in morning hours only.



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Immunoassay

Prolactin, Serum*

Date	17/Aug/2021 01:46PM	Unit	Bio Ref Interval
Prolactin CLIA	6.23	ng/mL	

Ref Range

Males : 2.64 - 13.13

Females :

Premenopausal

(<50 years of age): 3.34 - 26.74

age):

Postmenopausal

(>50 years of age): 2.74 - 19.64

age):

Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery
Decreased in pituitary apoplexy (Sheehan's Syndrome)

Kindly correlate with clinical findings

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